From early to fewer first births: ADHD and family formation among young adults

Sanna Kailaheimo-Björkqvist [1,2,3] Niina Metsä-Simola [1,3] Mikko Myrskylä [1,3,4]

► Additional supplemental material is published online only. To view, please visit the journal online (https://doi.org/10.1136/jech-2026-225863).

[1]Helsinki Institute for Demography and Population Health, University of Helsinki, Helsinki, Finland
[2]INVEST Research Flagship Centre, University of Turku, Turku, Finland
[3]The Max Planck—University of Helsinki Center for Social Inequalities in Population Health (MaxHel Center), University of Helsinki, Helsinki, Finland
[4]Max-Planck-Institute for Demographic Research, Rostock, Germany

Correspondence to
Dr Sanna Kailaheimo-Björkqvist; sanna.kailaheimo-bjorkqvist@helsinki.fi

Received 8 January 2026
Accepted 2 June 2026
Published Online First 23 June 2026

Check for updates

© Author(s) (or their employer(s)) 2026. No commercial re-use. See rights and permissions. Published by BMJ Group.

To cite: Kailaheimo-Björkqvist S, Metsä-Simola N, Myrskylä M. *J Epidemiol Community Health* 2026;**80**:784–789.

ABSTRACT

Background Attention-deficit/hyperactivity disorder (ADHD) is common and associated with relationship difficulties, which may influence entry into parenthood.

Methods We used Finnish population register data to follow 759,430 individuals born in 1982–1993 and examined how ADHD is associated with the likelihood and timing of a first birth. ADHD was identified using healthcare diagnoses and prescription records. We estimated discrete-time event history models and sibling fixed-effects models to account for shared family background. All analyses were conducted separately for women and men.

Results In models adjusted for age and birth cohort, ADHD was associated with a lower likelihood of a first birth (ages 18–38) (men OR 0.92; women OR 0.90). After adjusting for partnership characteristics, the association reversed (men OR 1.07; women OR 1.09). All age-stratified analyses showed that ADHD was associated with a higher likelihood of first birth at ages 18–23, but with a lower likelihood at ages 24–30 and 31–38, indicating earlier entry into parenthood among individuals with ADHD. Results from sibling fixed-effects models were broadly similar to the population-level estimates, suggesting limited confounding by shared family background.

Conclusions ADHD is associated not only with earlier entry into parenthood but also with a reduced overall probability of having a first child. Accounting for partnership status is important, as it largely explains the negative association observed in age-adjusted models. These findings highlight partnerships as a key pathway linking ADHD to fertility outcomes.

WHAT IS ALREADY KNOWN ON THIS TOPIC

⇒ Attention-deficit/hyperactivity disorder (ADHD) is linked to relationship difficulties and early sexual behaviour, but evidence on fertility patterns and timing is limited.

WHAT THIS STUDY ADDS

⇒ Individuals with ADHD are less likely to have children overall, but those who do become parents earlier. Accounting for partnerships reverses the association with first births, highlighting partnership dynamics as a key mechanism.

HOW THIS STUDY MIGHT AFFECT RESEARCH, PRACTICE OR POLICY

⇒ Supporting relationship stability and reproductive health among individuals with ADHD may help reduce inequalities in family formation.

INTRODUCTION

Investigating how attention-deficit/hyperactivity disorder (ADHD) is related to adulthood outcomes is societally relevant since ADHD is among the most common neurodevelopmental disorders, affecting an estimated 5%–10% of school-aged children and 2%–5% of adults.[1] Neurodevelopmental disorder diagnoses, ADHD in particular, have become increasingly prevalent in recent decades. Prior research on ADHD has focused on various outcomes—such as treatment, life satisfaction, criminality, labour-market and educational attachment[2–8]—but surprisingly, its links to fertility have not yet been investigated.

ADHD diagnoses are more common among boys, who are also diagnosed earlier than girls.[9] Symptom presentation differs by gender: boys more often display hyperactivity, impulsivity and other externalising behaviours, whereas girls tend to show internalising symptoms such as anxiety and depression as well as academic underachievement.[10] Individuals with ADHD often experience greater difficulties in peer and social relationships, with some studies suggesting stronger impairments among women.[11 12] Individuals with ADHD are more likely than their peers to engage in risk behaviours such as smoking, have a lower age at first intercourse and experience teenage pregnancy,[8 13] which suggests that ADHD may be related to earlier age at first birth and having a child outside the context of coresidential partnerships. Although ADHD has been linked to various aspects of social functioning, its relationship to childbearing remains largely underexplored. Together, previous research suggests that ADHD may influence not only whether individuals have children but also the timing of first birth.

Core ADHD-related challenges—such as difficulties with executive functioning, attention and impulse control—can lead to forgetfulness and disorganisation, which often strain partnerships and increase the risk of separation.[14 15] These challenges may also increase the likelihood of remaining unpartnered. Previous studies have examined ADHD in romantic relationships, documenting lower relationship satisfaction, more frequent conflict and higher risks of separation among individuals with ADHD compared with those without.[16–20] Couples in which at least one partner has ADHD are also more likely to experience stress and depression than couples without ADHD.[21] This

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.

leads to a higher likelihood of union instability among individuals with ADHD, and such relationship instability may, in turn, reduce opportunities for planned childbearing and contribute to childlessness.

In this study, we extend previous research by examining how ADHD is related to the likelihood and timing of a first birth. We also assess the role of partnership status and a partner's ADHD diagnosis in shaping these associations. To our knowledge, we are the first to examine the link between ADHD and family formation.

METHODS

Data

We use Finnish total population register data for cohorts born between 1982 and 1993 (371,563 women; 387,867 men). The sample is restricted to individuals born in Finland since we have complete linkage of education, healthcare and sociodemographic data throughout the study period for these persons. All methods were carried out in accordance with relevant guidelines and regulations. Informed consent is not required for register-based studies in Finland because participants are not contacted. The register-based data cannot be shared due to legal restrictions, but access can be applied for through Statistics Finland and THL.

The main explanatory variable is ADHD, and it is measured using both diagnostic information from specialised healthcare registers and data on prescription medication purchases. The ADHD diagnostic information (ICD-10 codes, F90) is derived from the specialised healthcare registers and the medication information (ATC-codes, NO6B) from the prescription register of Social Insurance Institution of Finland. All psychotropic medication is prescribed by clinical doctors, and all persons residing in Finland are entitled to reimbursement for medication expenses by the Social Insurance Institution of Finland, thus increasing the validity of our measure. The partner's disorders are measured in the same way as the index person's disorders. In the analysis, ADHD is treated as a time-invariant variable because diagnostic criteria require that symptoms were present already in childhood. The outcome variable is a time-varying indicator of first birth, measured as the occurrence of a first child in a given year between ages 18 and 38.

The control variables are year of birth, age, number of partners, own education and current partnership status. All control variables are fully time varying. Year of birth is a categorical variable and varies from 1982 to 1993. Age is a categorical variable and has values from 18 to 38. Own highest (achieved) education was categorised into basic, secondary (including vocational and general tracks) and tertiary education (including those with a bachelor's degree or higher). The current partnership status was categorised into having a partner without ADHD, having a partner with ADHD and not having a partner. Partnerships are defined as coresidential partnerships, combining both marriages and cohabitations. Number of partners was the highest partner order at each observation point.

METHODS

We estimated discrete-time event history models of first birth using logistic regression and person-year data, with age as the time scale. Individuals were followed from age 18 until the first birth or censoring. The outcome was a time-varying binary indicator for having a first child in year t, predicted by time-invariant ADHD status. All time-varying covariates were lagged by 1 year (t–1). Education was measured as the highest level completed by the end of year t–1.

All analyses were conducted separately for men and women. We estimated both pooled models covering ages 18–38 and age-stratified models for ages 18–23, 24–30 and 31–38. To examine potential mechanisms, we fitted stepwise models: model 1 adjusted for age and year of birth; model 2 additionally adjusted for educational attainment; and model 3 further adjusted for current partnership status and number of partners. Results from these models are presented as ORs. To examine age-related heterogeneity in more detail, we additionally estimated interaction models between ADHD and age across ages 18–38.

To assess whether the observed associations were driven by shared family background, we additionally estimated sibling fixed-effects discrete-time event history models. These models compared full siblings within the same family and controlled for all time-invariant characteristics shared by siblings, including early-life familial and socioeconomic factors. The sibling fixed-effects analyses include all full siblings from families with two or more children. Identification is based on within-family differences, meaning that only sibships in which siblings differ in ADHD status contribute to the estimation of the ADHD effect. The sibling fixed-effects models were estimated using conditional logistic regression with family-level strata.

All analyses were conducted using Stata V.17.0. The main analysis code is available on GitHub.

RESULTS

The descriptive results are reported at the end of the observation period, that is, in 2020, when individuals were aged 27–38. Young adults with ADHD were less likely to have children, had lower levels of education and were less often in partnerships (table 1). ADHD was associated with a roughly 10 percentage-point higher prevalence of childlessness among both men and women, and with an age at first birth approximately 2 years younger compared with individuals without ADHD.

Models 1–2 capture the overall association between ADHD and first birth, including pathways operating through partnership formation and stability (table 2). Model 3, which conditions on current partnership status and number of prior partners, can be interpreted as estimating the association under a hypothetical scenario in which individuals with and without ADHD have similar partnership trajectories. Across ages 18–38, ADHD is associated with lower first-birth odds in models that adjust only for age and cohort (men M1 OR=0.918, 95% CI 0.895 to 0.942; women M1 OR=0.900, 95% CI 0.873 to 0.928). Adding education explains part of this negative association (men M2 OR=0.941; women M2 OR=0.921). After additionally accounting for current partnership status and number of prior partners, the association reverses and becomes positive overall (men M3 OR=1.071, 95% CI 1.044 to 1.098; women M3 OR=1.089, 95% CI 1.059 to 1.120). This reversal indicates that conditional on similar partnership histories, individuals with ADHD would have a higher likelihood of entering parenthood. These findings suggest that ADHD is associated with lower rates of partnership formation and stability, contributing to a lower overall probability of first birth.

Next, we estimated models stratified by age group. At ages 18–23, ADHD is linked to higher first-birth odds in all models (men M3 OR=1.324, 95% CI 1.243 to 1.411; women M3 OR=1.369, 95% CI 1.308 to 1.433). At ages 24–30, the association is negative in all models for women (M3 OR=0.928, 95% CI 0.882 to 0.976), whereas for men it changes from negative in M1 and M2 (OR=0.82–0.85) to slightly positive (but not statistically significant) in M3 (OR=1.023, 95% CI 0.976

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.

Table 1 Descriptive statistics by ADHD status and gender at the end of the observation period (women N=371,563; men N=387,867)

		Women (%)		Men (%)	
		No ADHD	ADHD	No ADHD	ADHD
ADHD			1.7		2.1
Child		57	50	44	36
Highest education					
	basic	6	17	11	25
	secondary	43	56	55	58
	tertiary	51	27	34	17
Partnership					
	partner, no ADHD	64	39	55	34
	partner with ADHD	1	3	1	2
	no partner	35	57	44	64
		Mean (SD)		Mean (SD)	
Number of partners		1.0 (1.0)	0.8 (1.1)	0.8 (0.9)	0.6 (0.9)
Age at first birth		26.7 (4.5)	24.3 (4.5)	27.9 (4.3)	26.4 (4.4)
Number of children		1.2 (1.3)	1.0 (1.3)	0.8 (1.2)	0.7 (1.2)

Min/Max for number of partners is 0 and 9 in all categories. Age at first birth varies from 18 to 38, and number of children from 0 to 9. Age is treated as a categorical variable in the models.
ADHD, attention-deficit/hyperactivity disorder.

to 1.072). At ages 31–38, ADHD is associated with lower first-birth odds for both genders, with a stronger association among women (men M3 OR=0.876, 95%CI 0.799 to 0.961; women M3 OR=0.665, 95%CI 0.592 to 0.748).

Thus, ADHD is associated with earlier transitions into parenthood, alongside lower first-birth probabilities at typical peak ages. This pattern persists after full adjustment, although it is attenuated, indicating that partnership dynamics account for a substantial share of the observed differences. In contrast to the reversal observed in the pooled models after adjusting for partnership characteristics, the age-stratified estimates do not reverse but instead show a consistent age gradient. Overall, the results indicate higher first-birth odds at younger ages and lower odds at older ages among individuals with ADHD (see also figure 1).

To examine the magnitude of the association between ADHD and first birth by age, we used AMEs and conducted an interaction analysis between own ADHD and age (figure 1). The results show elevated probabilities of having a first birth at younger ages and reduced probabilities at later ages for men and women alike. In the age-adjusted and cohort-adjusted model (M1), ADHD is positively related to first birth in the late teens and early 20s, peaks around ages 20–22, and then declines, turning negative from the mid-20s to late 20s. The negative association is largest around ages 28–31, after which it attenuates towards zero in the mid-30s to late 30s.

Robustness checks and additional analyses

To assess the role of shared family background, we estimated sibling fixed-effects discrete-time event history models comparing full siblings within the same family. The results were broadly similar to the population-level models, indicating that shared

Table 2 ADHD and first birth

	Men			Women		
	M1	M2	M3	M1	M2	M3
ADHD						
Age 18–38	0.918***	0.941***	1.071***	0.900***	0.921***	1.089***
	(0.895, 0.942)	(0.917, 0.966)	(1.044, 1.098)	(0.873, 0.928)	(0.893, 0.950)	(1.059, 1.120)
Age 18–23	1.509***	1.267***	1.324***	1.688***	1.305***	1.369***
	(1.414, 1.612)	(1.187, 1.352)	(1.243, 1.411)	(1.611, 1.768)	(1.247, 1.366)	(1.308, 1.433)
Age 24–30	0.820***	0.846***	1.023	0.691***	0.780***	0.928**
	(0.782,0.861)	(0.806,0.888)	(0.976,1.072)	(0.657,0.727)	(0.741,0.821)	(0.882,0.976)
Age 31–38	0.596***	0.696***	0.876**	0.444***	0.574***	0.665***
	(0.543, 0.654)	(0.634, 0.763)	(0.799, 0.961)	(0.395, 0.499)	(0.511, 0.645)	(0.592, 0.748)
Age dummy	X	X	X	X	X	X
Year of birth	X	X	X	X	X	X
Education		X	X		X	X
Current partnership status			X			X
Number of partners			X			X

ORs (95% CIs) for the association between own ADHD and having a first birth from discrete-time event-history models by genders, shown for the pooled age range (18–38) and for age-stratified models (18–23, 24–30, 31–38).
*p < 0.05, **p < 0.01, ***p < 0.001.
ADHD, attention-deficit/hyperactivity disorder.

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.

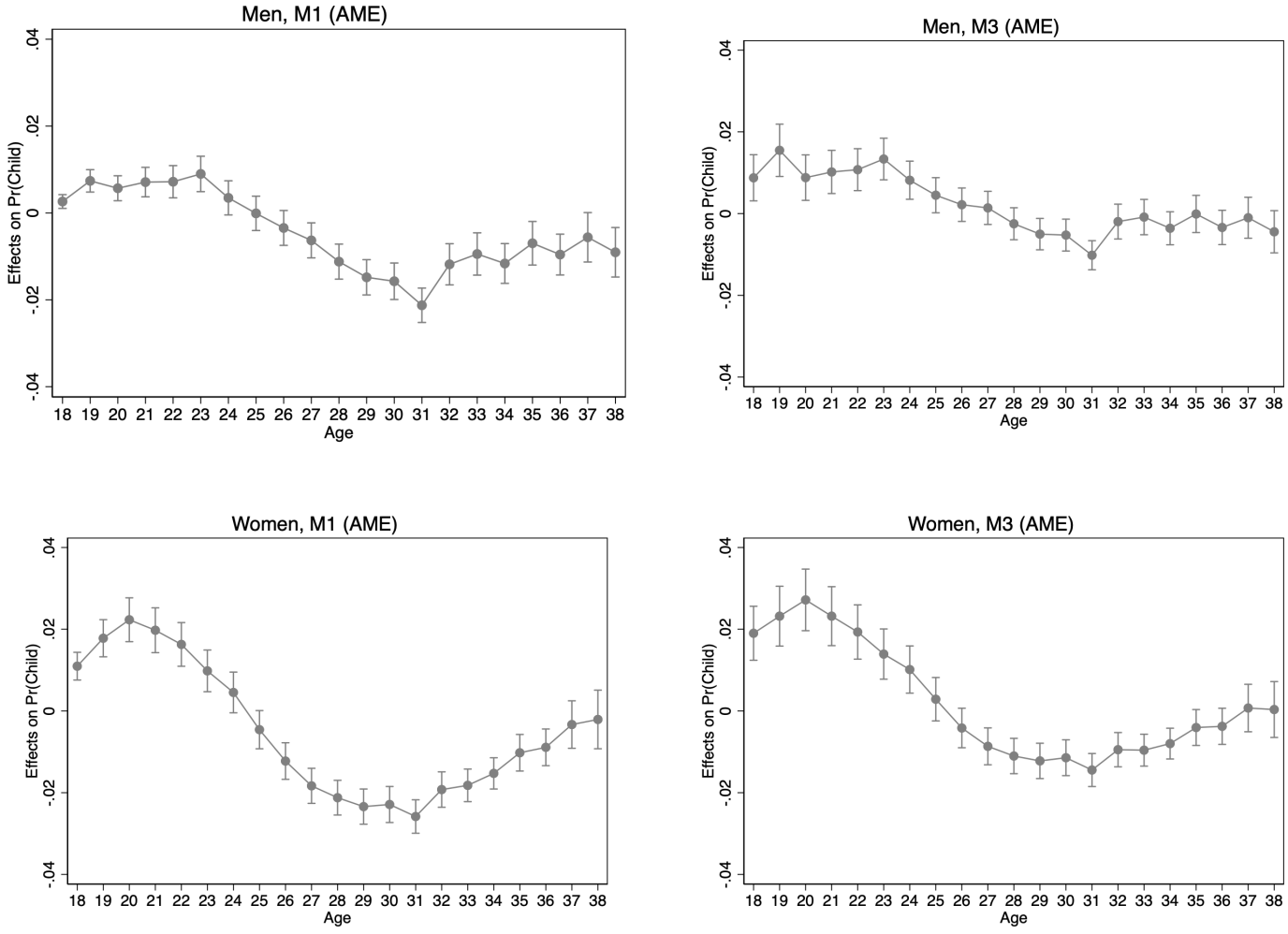


Figure 1 Average marginal effects (AMEs) of own ADHD on the probability of a first birth by age (interaction between own ADHD and age). M1 controls for year of birth. M3 is the fully adjusted model (adding education, current partnership status and number of partners). Points are AMEs with 95% CIs. ADHD, attention-deficit/hyperactivity disorder.

family-level confounding explains only a limited part of the association between ADHD and entry into parenthood. Among men, ADHD remained positively associated with the likelihood of a first birth in the fully adjusted sibling fixed-effects model, whereas among women the association was attenuated and no longer statistically significant (online supplemental appendix table A1 and figure A1).

We also conducted a robustness analysis by examining whether the results remained similar when ADHD was measured for all individuals by age 27. The results were largely unchanged, with only a slight strengthening of the association (online supplemental appendix table A2 and figure A2).

To explore how partner characteristics are associated with entry into parenthood, we further estimated age-stratified models including interactions between own ADHD status and current partnership status (no partner, partner without ADHD, partner with ADHD). This approach was motivated by the possibility that the association between ADHD and first birth may be amplified when both partners have ADHD (c.f. previous findings on mental health disorders).[22 23] However, analyses stratified by partner ADHD status did not indicate meaningful effect modification (online supplemental appendix figure A3).

DISCUSSION

This study examined how ADHD is associated with the likelihood and timing of first births among young adults using Finnish total population register data. The results show that young adults with ADHD are less likely to have a first birth between ages 18 and 38 but controlling for partnership status reverses this association. These findings are consistent with previous studies showing that ADHD is associated with interpersonal difficulties and relationship instability[17 18] and underscores the role of partnerships as a key mechanism connecting ADHD to family formation outcomes. Furthermore, although individuals with ADHD were less likely to have a first birth overall, those who became parents did so at younger ages, suggesting less postponement and potentially a greater role of less planned childbearing, including births outside stable partnerships. Some support for this interpretation comes from our additional analyses, which indicate that individuals with ADHD may in some cases have a higher likelihood of having children outside coresidential partnerships. This pattern is consistent with research linking ADHD to impulsivity, earlier sexual debut and an increased likelihood of teenage pregnancy.[8 13]

Sibling fixed-effects analyses further showed that these associations were broadly similar when comparing full siblings within the same family, suggesting that shared family background

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.

explains only a limited part of the observed associations. This strengthens the interpretation that individual-level life-course processes, rather than unobserved family-level factors, play a central role in shaping fertility outcomes among individuals with ADHD.

Robustness analyses confirmed the stability of our findings. Restricting ADHD measurement to diagnoses made by age 27 produced nearly identical results. In addition, the consistency between population-level and sibling fixed-effects estimates supports the robustness of the results to confounding by shared family background. Together, these analyses strengthen the conclusion that partnership formation and stability are key pathways in the relationship between ADHD and first births.

The strengths of this study include the use of comprehensive nationwide register data, measurement of ADHD based on healthcare diagnoses and medication records, and the ability to link individual and partner characteristics. Nonetheless, several limitations should be acknowledged. ADHD prevalence in these cohorts reflects the diagnostic practices of the time. However, our additional analyses comparing early and late cohorts did not indicate systematic attenuation of associations (available from author).

ADHD is measured as a binary variable in our data, and we were, therefore, unable to examine variation in severity even though ADHD represents a continuous spectrum of symptoms and functional impairment. In addition, while partnership status and number of partners were included, information on partnership quality or informal relationships such as living-apart-together unions was not available.

Finally, our analyses capture fertility outcomes only up to ages 27–38 and, therefore, do not reflect completed fertility, as follow-up ends at age 38 at the latest and earlier for younger cohorts. As the age at first birth has increased across cohorts, measures focusing on early adulthood increasingly reflect differences in the timing of childbearing rather than completed fertility. Future research should examine whether the associations observed here persist across the full reproductive life course once these cohorts reach older ages, particularly given that ADHD has become increasingly common in more recent cohorts.

Overall, this study highlights partnership dynamics as a central mechanism through which ADHD influences family formation. The results show that young adults with ADHD are less likely to have children overall, but those who become parents do so at a younger age than their peers without ADHD. By showing that these patterns persist after accounting for shared family background, the findings underscore the importance of relationship trajectories and life-course timing in understanding fertility outcomes among individuals with ADHD. Supporting stable relationships and reproductive health among individuals with ADHD may help reduce fertility disparities. Understanding these dynamics is particularly relevant in light of the rising prevalence of ADHD.

Contributors SKB designed the study, conducted the analyses and drafted the manuscript. NMS contributed to the study design, interpretation of the results and revision of the manuscript. MM contributed to the interpretation and revision of the manuscript. All authors approved the final version. SKB is the guarantor of this study.

Funding SKB was supported by the Strategic Research Council (SRC), FLUX consortium, decision numbers 364374 and 364375, Antti and Jenny Wihuri foundation, and partially supported by the INVEST Research Flagship Centre (Research Council of Finland, decision number: 369118). NMS was supported by the Research Council of Finland (grant #363724). MM was supported by the European Research Council under the European Union's Horizon 2020 research and innovation programme (grant agreement number 101019329); the SRC, FLUX consortium, decision numbers 364374 and 364375; by grants to the Max Planck—University of Helsinki Center from the Max Planck Society (5714240218); Jane and Aatos Erkko Foundation (210046); Faculty of Social Sciences at the University of Helsinki (77204227); the Cities of Helsinki, Vantaa and Espoo; and the European Union (ERC Synergy, BIOSFER, 101071773). The views and opinions expressed in the article are, however, those of the authors only and do not necessarily reflect those of the European Union or the European Research Council. Neither the European Union nor the granting authority can be held responsible for them.

Competing interests None declared.

Patient consent for publication Not applicable.

Ethics approval The use of register data for this study was approved by the Ethics Committee of Statistics Finland (TK/2182/07.03.00/2024) and the Finnish Institute for Health and Welfare (THL/3141/6.02.00/2022). All methods were carried out in accordance with relevant guidelines and regulations. Informed consent is not required for register-based studies in Finland because participants are not contacted. The register-based data cannot be shared due to legal restrictions, but access can be applied for through Statistics Finland and THL.

Provenance and peer review Not commissioned; externally peer-reviewed.

Data availability statement No data are available. The register-based data cannot be shared due to legal restrictions, but access can be applied for through Statistics Finland and THL.

Supplemental material This content has been supplied by the author(s). It has not been vetted by BMJ Publishing Group Limited (BMJ) and may not have been peer-reviewed. Any opinions or recommendations discussed are solely those of the author(s) and are not endorsed by BMJ. BMJ disclaims all liability and responsibility arising from any reliance placed on the content. Where the content includes any translated material, BMJ does not warrant the accuracy and reliability of the translations (including but not limited to local regulations, clinical guidelines, terminology, drug names and drug dosages), and is not responsible for any error and/or omissions arising from translation and adaptation or otherwise.

ORCID iDs
Sanna Kailaheimo-Björkqvist https://orcid.org/0000-0001-5238-0277
Niina Metsä-Simola https://orcid.org/0000-0001-8813-3980

REFERENCES

1 Ornoy A, Koren G. The Effects of Drugs used for the Treatment of Attention Deficit Hyperactivity Disorder (ADHD) on Pregnancy Outcome and Breast-feeding: A Critical Review. *Curr Neuropharmacol* 2021;19:1794–804.
2 Ostinelli EG, Schulze M, Zangani C, *et al*. Comparative efficacy and acceptability of pharmacological, psychological, and neurostimulatory interventions for ADHD in adults: a systematic review and component network meta-analysis. *Lancet Psychiatry* 2025;12:32–43.
3 Türk S, Korfmacher A-K, Gerger H, *et al*. Interventions for ADHD in childhood and adolescence: A systematic umbrella review and meta-meta-analysis. *Clin Psychol Rev* 2023;102:102271.
4 Carlander A, Rydell M, Kataoka H, *et al*. A Remedy for Crime? A Systematic Review on the Effects of Pharmacological ADHD Treatment on Criminal Recidivism and Rehabilitation in Inmates With ADHD. *Brain Behav* 2024;14:e70120.
5 Christiansen MS, Labriola M, Kirkeskov L, *et al*. The impact of childhood diagnosed ADHD versus controls without ADHD diagnoses on later labour market attachment-a systematic review of longitudinal studies. *Child Adolesc Psychiatry Ment Health* 2021;15:34.
6 Chen L, Mittendorfer-Rutz E, Björkenstam E, *et al*. Labour market integration among young adults diagnosed with attention-deficit/hyperactivity disorder (ADHD) at working age. *Psychol Med* 2024;54:148–58.
7 French B, Nalbant G, Wright H, *et al*. The impacts associated with having ADHD: an umbrella review. *Front Psychiatry* 2024;15:1343314.
8 Soler Artigas M, Sánchez-Mora C, Rovira P, *et al*. Mendelian randomization analysis for attention deficit/hyperactivity disorder: studying a broad range of exposures and outcomes. *Int J Epidemiol* 2023;52:386–402.
9 Cheng A, Illango A, El-Kadi A, *et al*. Factors Impacting Gender Diagnostic Differences in ADHD: A Review. *The CHILD Journal* 2022;1.
10 Antoniou E, Rigas N, Orovou E, *et al*. ADHD Symptoms in Females of Childhood, Adolescent, Reproductive and Menopause Period. *Mater Sociomed* 2021;33:114:114–8:.
11 Ros R, Graziano PA. Social Functioning in Children With or At Risk for Attention Deficit/Hyperactivity Disorder: A Meta-Analytic Review. *Journal of Clinical Child & Adolescent Psychology* 2018;47:213–35.
12 Kok FM, Groen Y, Fuermaier ABM, *et al*. Problematic Peer Functioning in Girls with ADHD: A Systematic Literature Review. *PLoS One* 2016;11:e0165119.
13 Meinzer MC, LeMoine KA, Howard AL, *et al*. Childhood ADHD and Involvement in Early Pregnancy: Mechanisms of Risk. *J Atten Disord* 2020;24:1955–65.
14 Hankin CS. ADHD and its impact on the family. *Drug Benefit Trends* 2001;13:15–6.
15 Robin AL, Payson E. The Impact of ADHD on Marriage. *ADHD Rep* 2002;10:9–14.
16 Huynh-Hohnbaum ALT, Benowitz SM. Effects of adult ADHD on intimate partnerships. *J Fam Soc Work* 2022;25:169–84.

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.

17 Wymbs BT, Canu WH, Sacchetti GM, *et al*. Adult ADHD and romantic relationships: What we know and what we can do to help. *J Marital Fam Ther* 2021;47:664–81.
18 Katzman MA, Bilkey TS, Chokka PR, *et al*. Adult ADHD and comorbid disorders: clinical implications of a dimensional approach. *BMC Psychiatry* 2017;17:302.
19 Klein RG, Mannuzza S, Olazagasti MAR, *et al*. Clinical and functional outcome of childhood attention-deficit/hyperactivity disorder 33 years later. *Arch Gen Psychiatry* 2012;69:1295–303.
20 Michielsen M, Comijs HC, Aartsen MJ, *et al*. The relationships between ADHD and social functioning and participation in older adults in a population-based study. *J Atten Disord* 2015;19:368–79.
21 Ghahramanzadeh M, Mashhadi A, Shamloo ZS, *et al*. Comparing emotional disturbances and quality of life in couples with attention-deficit and hyperactivity disorder and ordinary couples. *Journal of Fundamentals of Mental Health* 2021;23.
22 Kailaheimo-Lönnqvist S, Moustgaard H, Martikainen P, *et al*. Own depression, partner's depression, and childlessness: A nationwide register-based study. *Social Science & Medicine* 2024;361:117356.
23 Kailaheimo-Björkqvist S, Nisén J, Metsä-Simola N, *et al*. n.d. Linking mental health disorders to childlessness: The roles of disorder type and partnership. *JFamRes*38:130–47.

J Epidemiol Community Health: first published as 10.1136/jech-2026-225863 on 23 June 2026. Downloaded from http://jech.bmj.com/ on October 21, 2026 at SUNY Stony Brook NERL Consortia . Protected by copyright, including for uses related to text and data mining, AI training, and similar technologies.